

- a) secondary malignant neoplasm ([2D50-2E2Z](#) *Malignant neoplasm metastases*) as due to another malignant neoplasm or neoplasms of the central nervous system [2A00-2A0Z](#) (see section [2.21.5] (#Malignant#neoplasms) to ensure proper coding).
- b) the following malignant neoplasms as due to HIV disease ([1C60](#) - [1C62](#)) :
- [2A60.5](#) Blastic plasmacytoid dendritic cell neoplasm, *specified as* primary in brain
 - [2A80](#) Follicular lymphoma, *specified as* primary in brain
 - [2A81](#) Diffuse large B-cell lymphomas, *specified as* immunoblastic
 - [2A85.5](#) Mantle cell lymphoma, *specified as* primary in brain
 - [2A85.6](#) Burkitt lymphoma including Burkitt leukaemia
 - [2A86](#) B-cell lymphoma, mixed features, *specified as* primary in brain
 - [2A87](#) Mature B-cell neoplasms, unspecified, *specified as* primary in brain
 - [2A90-2B2Z](#) Mature T-cell or NK-cell neoplasms, *specified as* primary in brain
 - [2B30](#) Hodgkin lymphoma, *specified as* primary in brain
 - [2B57](#) Kaposi sarcoma, primary site
 - [2B6A](#) Malignant neoplasms of oropharynx
 - [2C00](#) Malignant neoplasms of anus or anal canal
 - [2C70](#) Malignant neoplasms of vulva
 - [2C71](#) Malignant neoplasms of vagina
 - [2C77](#) Malignant neoplasms of cervix uteri, *specified as* invasive
 - [2C81](#) Malignant neoplasms of penis

Consequence condition

Malignant neoplasm of oropharynx etc., listed above

Malignant neoplasms not listed above

Causal condition

Accept HIV diseases

Do not accept other causes

Do not accept other causes

2.19.1.4 Congenital or constitutional haemorrhagic condition due to other conditions

Do not accept congenital or constitutional haemorrhagic condition [3B10-3B1Z](#) as due to any other cause.

Consequence condition

Congenital or constitutional haemorrhagic condition

Causal condition

Do not accept other causes

2.19.1.5 Anaphylaxis due to external causes

Do not accept Anaphylaxis ([4A84](#)) as due to any other causes except Intentional self-harm ([PB80-PD3Z](#)) or Assault ([PD50-PF2Z](#)).

Consequence Caused by

Anaphylaxis **Accept** Intentional self-harm or Assault
Do not accept other causes

2.19.1.6 Diabetes due to other conditions

Do not accept Type 1 diabetes mellitus as due to any other cause except conditions causing autoimmune destruction of beta-cells.

Do not accept Type 2 diabetes mellitus as due to any other cause except conditions causing insulin resistance.

Do not accept 'Other and Unspecified diabetes mellitus' as due to any other cause except conditions causing damage to the pancreas.

See Mortality Annex [3.14.8](#) for a list of the conditions that can cause diabetes.

Consequence condition**Causal condition**

Type 1 diabetes mellitus

Accept conditions causing autoimmune destruction of beta cells

Do not accept other causes

Type 2 diabetes mellitus

Accept conditions causing insulin resistance

Do not accept other causes

Other and Unspecified diabetes mellitus

Accept conditions causing damage to the pancreas

Do not accept other causes

2.19.1.7 Rheumatic fever due to other conditions

Do not accept Acute rheumatic fever (1B40-1B42), Heart valve diseases (BB60-BC0Z) with fifth character .0 rheumatic (if the fifth character is available), and [BC20](#) Chronic rheumatic heart diseases, not elsewhere classified due to other cause, except:

- [1B50](#) Scarlet fever
- [1B51](#) Streptococcal pharyngitis
- [CA03.0](#) Streptococcal tonsillitis

Consequence condition	Causal condition
Acute rheumatic fever	Accept
Rheumatic heart valve diseases	- Scarlet fever
Chronic rheumatic heart diseases NEC	- Streptococcal pharyngitis
	- Acute tonsillitis
	Do not accept other causes

2.19.1.8 Hypertension due to other conditions

Do not accept hypertensive conditions as due to a neoplasm, **except**:

- [Endocrine neoplasms](#)
- [Renal neoplasms](#)
- [Carcinoid tumours](#)

Consequence condition	Causal condition
Hypertensive conditions	Accept
	- endocrine neoplasms
	- renal neoplasms
	- carcinoid tumours
	Do not accept other neoplasms

2.19.1.9 Certain ischaemic heart disease due to other conditions

Do not accept Angina pectoris ([BA40](#)) and Chronic ischaemic heart disease ([BA50-BA5Z](#)) or Coronary atherosclerosis ([BA52](#)) as due to a neoplasm.

Consequence condition	Causal condition
Angina pectoris	Accept other causes
Chronic ischaemic heart disease	
Coronary atherosclerosis	Do not accept neoplasms

2.19.1.10 Atherosclerosis due to other conditions

Do not accept an atherosclerotic condition as due to a neoplasm.

Consequence condition	Causal condition
An atherosclerotic condition	Accept other causes
	Do not accept neoplasms

2.19.1.11 Developmental anomalies due to other conditions

Do not accept [Developmental anomalies] as due to any other cause, including immaturity, except:

- developmental anomalies due to a chromosome abnormality or a congenital malformation syndrome
- Congenital hypoplasia of lung ([LA75.2](#)) due to a congenital anomaly

Consequence condition	Causal condition
A developmental anomaly	Accept chromosome abnormality, congenital malformation syndrome Do not accept other causes, including immaturity
Congenital hypoplasia of lung	Accept a developmental anomaly Do not accept other causes, including immaturity

2.19.1.12 Unintentional cause of morbidity or mortality due to other conditions

Do not accept unintentional cause of morbidity or mortality as due to causes coded in other chapters, except:

- Fall or a fracture as due to a 'Certain specified disorders of bone density or structure' or 'Low bone mass disorders'
- Fall as due to a (pathological) fracture caused by 'Certain specified disorders of bone density or structure' or 'Low bone mass disorders'
- Inhalation or aspiration as due to other causes

Consequence condition

Unintentional cause of morbidity or mortality [PA00-PB6Z](#) *not listed below*

Fall [PA60-PA6Z](#)

[PB6Z](#) , *specified as* fracture

[PB04](#) Unintentional threat to breathing by inhalation or ingestion of gastric contents

[PB05](#) Unintentional threat to breathing by inhalation or ingestion of liquids

[PB06](#) Unintentional threat to breathing by inhalation or ingestion of food

[PB07](#) Unintentional threat to breathing by inhalation or ingestion of other objects or materials

Causal condition

Do not accept causes in other chapters

Accept [FB80](#) or [FB83](#)

Accept a (pathological) fracture caused by [FB80](#) or [FB83](#)

Do not accept other causes in other chapters

Accept [FB80](#) or [FB83](#)

Do not accept other causes in other chapters

Accept other causes

Accept other causes

Accept other causes

Accept other causes

2.19.1.13 Suicide due to other conditions

Do not accept suicide ([PB80-PD3Z](#)) as due to any other cause.

Consequence condition	Causal condition
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Suicide	Do not accept other causes
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2.19.1.14 Obstetric conditions due to other conditions

Do not accept Ectopic pregnancy ([JA01](#)) and Molar pregnancy ([JA02](#)) as due to other causes.

Consequence condition	Causal condition
------------------------------	-------------------------

JA01 Ectopic pregnancy	Do not accept other causes
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JA02 Molar pregnancy	—
-----------------------------	---

Do not accept Hypertensive disorders in pregnancy, childbirth, or the puerperium (JA20-21, JA23-JA25) as due to other causes.

Consequence condition

JA20-JA21, JA23-JA25 Hypertensive disorders in pregnancy, childbirth, or the puerperium

Causal condition

Do not accept other causes

Do not accept Maternal care related to placenta praevia or low lying placenta ([JA8B](#)) as due to other causes.

Consequence condition

JA8B Maternal care related to placenta praevia or low lying placenta

Causal condition

Do not accept other causes

2.19.2 Special instructions on obvious cause (Step SP6)

This section lists conditions that should be considered an obvious cause of conditions selected as tentative starting point in Steps SP1 to SP5.

2.19.2.1 Complications of HIV

Infectious diseases and HIV

Consider HIV disease ([1C60-1C62](#)) and [MA14.0](#) *Laboratory evidence of human immunodeficiency virus* as an obvious cause of infectious diseases:

- [1A32](#) Cryptosporidiosis
- [1A33](#) Cystoisosporiasis
- [1B21](#) Infections due to non-tuberculous mycobacteria
- [EA5Y](#) Cutaneous involvement by other specified bacterial infection
- [8A45.02](#) Progressive multifocal leukoencephalopathy
- Herpes simplex infection, of skin or mucous membrane ([1F00.0](#)), disseminated ([1F00.3](#)), other ([1F00.Y](#)) or unspecified ([1F00.Z](#)), *specified as* chronic ulcers, bronchitis, pneumonia, or oesophagitis
- [1D82](#) Cytomegaloviral disease, **except** Cytomegaloviral hepatitis ([1D82.0](#)), and **except for** liver, spleen, lymph nodes
- [1F23.2](#) Candidosis of gastrointestinal tract, *specified as* oesophagus
- [1F23.31](#) Pulmonary candidosis
- [1F25](#) Coccidioidomycosis
- [1F2A](#) Histoplasmosis
- [1F27](#) Cryptococcosis
- [CA40.20](#) Pneumonia due to pneumocystis

Consider HIV disease ([1C60](#) - [1C62](#)), but not Laboratory evidence of human immunodeficiency virus ([MA14.0](#)), as an obvious cause of infectious diseases (Chapter 01) not listed above, except those listed in Section ([2.19.1.2.1](#)).

Malignant neoplasms and HIV

Consider both HIV disease ([1C60-1C62](#)) and Laboratory evidence of HIV ([MA14.0](#)) as obvious causes of the following malignant neoplasms:

- [2A80](#) Follicular lymphoma, *specified as* primary in brain
- [2A81](#) Diffuse large B-cell lymphomas, *specified as* immunoblastic
- [2A85.5](#) Mantle cell lymphoma, *specified as* primary in brain
- [2A85.6](#) Burkitt lymphoma including Burkitt leukaemia
- [2A86](#) B-cell lymphoma, mixed features, *specified as* primary in brain
- [2A8Z](#) Mature B-cell neoplasms, unspecified, *specified as* primary in brain
- [2B30](#) Hodgkin lymphoma, *specified as* primary in brain
- [2B57](#) Kaposi sarcoma, primary site
- [2C77](#) Malignant neoplasms of cervix uteri, *specified as* invasive

Immune deficiency and HIV

Consider HIV disease ([1C60-1C62](#)) as an obvious cause of immune deficiency.

Pneumonia and HIV

Consider HIV disease ([1C60-1C62](#)) as an obvious cause of pneumonia ([CA40](#)).

Cachexia and HIV

Consider HIV disease ([1C60-1C62](#)) as an obvious cause of Cachexia, unspecified ([MG20.Z](#)).

2.19.2.2 Enterocolitis due to Clostridium difficile

Consider enterocolitis due to Clostridium difficile ([1A04](#)) as an obvious consequence of ([Drugs, medicaments or biological substances associated with injury or harm in therapeutic use] ([PLO0](#)), *specified as* antibiotic therapy.

2.19.2.3 Sepsis

Consider the following as obvious causes of sepsis ([1G40-1G41](#)):

- [Conditions that impair the immune system](#)
- [Wasting Diseases](#)
- [Diseases causing paralysis - sepsis](#)
- [Serious respiratory conditions](#)
- [Serious injuries](#) (grade 1–4 according to the injury priority list in the Mortality Annex ([3.14.5](#))).
- [Diseases of infectious origin](#) (this includes all the infections across the classification such as meningitis, peritonitis, osteomyelitis, pancreatitis, etc.)

2.19.2.4 Complications of diabetes

Consider Diabetes mellitus ([5A10-5A14](#)) as an obvious cause of the following conditions:

- [5C73](#) Acidosis
- [8C0Z](#) Polyneuropathy, unspecified
- [8C12](#) Certain specified mononeuropathies
- [8C7Y](#) Other specified primary disorders of muscles, *specified as* amyotrophy but without specification of aetiology
- [8D8Z](#) Disorders of autonomic nervous system, unspecified
- [9A96.Z](#) Anterior uveitis, unspecified
- [9B10.Z](#) Cataract, unspecified
- [9B65.Z](#) Chorioretinal inflammation
- [9B74](#) Retinal vascular occlusions
- [9B78.1](#) Background retinopathy and retinal vascular changes
- [9B78.2](#) Other proliferative retinopathy
- [9B78.5](#) Retinal haemorrhage
- [9B7Z](#) Disorders of the retina, unspecified
- [BD40.0](#) Lower limb atherosclerosis
- [BD4Z](#) Chronic arterial occlusive disease, unspecified
- [EE80.1](#) Necrobiosis lipoidica
- [ME60.2](#) Ulcer of skin of uncertain nature, *specified as* lower limb
- [FA2Z](#) Inflammatory arthropathies, unspecified
- [MG30.5Z](#) Chronic neuropathic pain, unspecified
- [GB40](#) Nephritic syndrome
- [GB41](#) Nephrotic syndrome
- [GB4Z](#) Persistent proteinuria or albuminuria
- [GB61](#) Chronic kidney disease
- [GB6Z](#) Kidney failure, unspecified
- [MF54.0](#) Smooth contracted kidney
- [GC2Z](#) Diseases of the urinary system, unspecified, *specified as* kidney conditions
- [MC85](#) Gangrene
- [MB20.1](#) Coma
- [MA18.Y](#) Other specified abnormal findings of blood chemistry, *specified as* acetonæmia, azotaemia, or related conditions

2.19.2.5 Dehydration

Consider any intestinal infectious disease as an obvious cause of Volume depletion ([5C70](#)).

2.19.2.6 Dementia

Consider conditions that typically involve [Irreversible brain damage](#) as obvious causes of dementia if no other cause of the dementia is stated.

- [8A00.0](#) Parkinson disease
- [8A20](#) Alzheimer disease
- [8B00-8B2Z](#) Cerebrovascular diseases (Exclusions : [8B10](#) Transient ischaemic attack and [8B21](#) Cerebrovascular disease with no acute cerebral symptom)
- [8E44](#) Post anoxic brain damage
- [8E40-8E4A.Z](#) Other specified disorders of the nervous system (inclusion terms)
- [8B24.0](#) Anoxic-ischaemic encephalopathy
- [KA40.0](#) Intracranial laceration or haemorrhage due to birth injury
- [KA40.1](#) Cerebral oedema due to birth injury
- [KA40.Y](#) Other specified birth injury to central nervous system

Consider trisomy 21 (Down syndrome) ([LD40.0](#)) as an obvious cause of Dementia due to Alzheimer disease ([6D80](#)), Dementia, unknown or unspecified cause ([6D8Z](#)) or Alzheimer disease ([8A20](#)).

2.19.2.7 Disorders of intellectual development

Consider the following conditions as obvious causes of disorders of intellectual development ([6A00](#)):

- [8D64.2](#) Post haemorrhagic hydrocephalus, *specified as* neonatal
- [KA00-KA0Z](#) Fetus or newborn affected by maternal factors or by complications of pregnancy, labour or delivery
- [KA20](#) Disorders of newborn related to slow foetal growth or foetal malnutrition
- [KA21](#) Disorders of newborn related to short gestation or low birth weight, not elsewhere classified
- [KA40.0](#) Intracranial laceration or haemorrhage due to birth injury
- [KA40.1](#) Cerebral oedema due to birth injury
- [KA40.Z](#) Birth injury to central nervous system, unspecified
- [KA4Z](#) Birth injury, unspecified
- [KA61](#) Other bacterial infections of the newborn
- [KA62](#) Viral infection in the fetus or newborn
- [KA63](#) Fungal infection of fetus or newborn
- [KA64](#) Parasitic diseases in the fetus or newborn
- [KA6Y](#) Other specified infections of the fetus or newborn
- [KA6Z](#) Infections of the fetus or newborn, unspecified
- [KB20](#) Intrauterine hypoxia
- [KB21](#) Birth asphyxia
- [KA82](#) Intracranial nontraumatic haemorrhage of fetus or newborn
- [KA86](#) Neonatal kernicterus
- [KB00](#) Neonatal cerebral ischaemia
- [KB01](#) Periventricular cysts of newborn
- [KB02](#) Neonatal cerebral leukomalacia
- [KB03](#) Neonatal encephalopathy
- [KB04](#) Hypoxic ischaemic encephalopathy of newborn
- [KB05](#) Neonatal hydrocephalus
- [KB06](#) Neonatal seizures

2.19.2.8 Heart failure and unspecified heart disease

Consider [Other heart conditions](#) as obvious causes of Diseases of the myocardium or cardiac chambers, unspecified ([BC4Z](#)) and Heart failure ([BD10-BD1Z](#)).

2.19.2.9 Embolism

Consider [Venous thrombosis](#), [Phlebitis or thrombophlebitis](#), [Valvular heart disease](#), [Childbirth](#) or any [Operation](#) as obvious causes of diseases described as ‘[Embolic](#)’. However, there must be a clear route from the place where the thrombus formed and the place of the embolism.

2.19.2.10 Oesophageal varices

Consider the following liver diseases as obvious causes of Oesophageal varices ([DA26.0](#)):

- [1E51](#) Chronic viral hepatitis
- [DB92](#) Non-alcoholic fatty liver disease
- [DB93](#) Hepatic fibrosis or cirrhosis
- [DB94](#) Alcoholic liver disease
- [DB96.1](#) Primary biliary cholangitis
- [DB97.2](#) Chronic hepatitis, not elsewhere classified
- [DB98.0](#) Infarction of liver
- [DB98.1](#) Peliosis hepatis
- [DB98.6](#) Hepatic veno-occlusive disease
- [DB98.7](#) Portal hypertension
- [DB98.8](#) Passive congestion of liver
- [DB99.2](#) Hepatorenal syndrome
- [DB99.Y](#) Other diseases of liver
- [DB9Z](#) Diseases of liver, unspecified

2.19.2.11 Pneumonia

Consider the following as obvious causes of Pneumonia ([CA40](#)) or Pneumonitis due to solids and liquids ([CA71](#)) except those due to oils or essences ([CA71.1](#)):

- [Conditions that impair the immune system](#)
- [Wasting Diseases](#)
- [Diseases causing paralysis](#)
- [Serious respiratory conditions](#)
- [Conditions that affect the process of swallowing](#)
- Other diseases that limit the ability to care for oneself, including dementia and degenerative diseases of the nervous system and poisoning
- [Serious injuries](#) (grade 1–4 according to the injury priority list in the Mortality Annex ([3.14.5](#))).

2.19.2.12 Pulmonary oedema

Consider the following conditions as obvious causes of Pulmonary oedema ([CB01](#)):

- heart disease (including pulmonary heart disease)
- conditions affecting the lung parenchyma, such as:
 - lung infections
 - aspiration and inhalation
 - respiratory distress syndrome
 - high altitude
 - circulating toxins
- conditions causing fluid overload, such as:
 - kidney failure
 - hypoalbuminaemia
- congenital anomalies affecting the pulmonary circulation, such as:
 - congenital stenosis of pulmonary veins

2.19.2.13 Nephritic syndrome

Consider any streptococcal infection (scarlet fever, streptococcal sore throat, etc.) as the obvious cause of Nephritic syndrome ([GB40](#)) or Nephrotic syndrome ([GB41](#)).

2.19.2.14 Pyelonephritis

Consider any urinary obstruction from conditions such as hyperplasia of prostate or ureteral stenosis as the obvious cause of the following Renal tubulo-interstitial diseases:

- [GB50](#) Acute tubulo-interstitial nephritis
- [GB51](#) Acute pyelonephritis
- [GB54](#) Tubulo-interstitial nephritis, not specified as acute or chronic
- [GB55.Y](#) Other specified chronic tubulo-interstitial nephritis
- [GB55.Z](#) Chronic tubulo-interstitial nephritis, unspecified

2.19.2.15 Acute renal failure

Consider a urinary tract infection as an obvious cause of Acute kidney failure ([GB60](#)), provided there is no indication that the renal failure was present before the urinary tract infection developed.

2.19.2.16 Primary atelectasis of newborn

Consider the following congenital kidney conditions, fetus or newborn affected by premature rupture of membranes ([KA01.1](#)) or by oligohydramnios ([KA01.2](#)) as obvious causes of Primary atelectasis of newborn ([KB2B](#)):

- [GB82](#) Autosomal dominant tubulointerstitial disease
- [GB8Y](#) Other specified cystic or dysplastic kidney disease
- [GB8Z](#) Cystic or dysplastic kidney disease, unspecified
- [LB31.8](#) Atresia or stenosis of ureter
- [LB31.9](#) Agenesis of ureter
- [LB31.Y](#) Other specified structural developmental anomalies of urinary tract
- [LD2F.13](#) Meckel-Gruber syndrome

2.19.2.17 Premature rupture of membranes and oligohydramnios

Consider the following congenital kidney conditions as obvious causes of fetus or newborn affected by premature rupture of membranes ([KA01.1](#)) or by oligohydramnios ([KA01.2](#)):

- [GB82](#) Autosomal dominant tubulointerstitial disease
- [GB8Y](#) Other specified cystic or dysplastic kidney disease
- [GB8Z](#) Cystic or dysplastic kidney disease, unspecified
- [LB31.8](#) Atresia or stenosis of ureter
- [LB31.9](#) Agenesis of ureter
- [LB31.Y](#) Other specified structural developmental anomalies of urinary tract
- [LD2F.13](#) Meckel-Gruber syndrome

2.19.2.18 Haemorrhage

Consider anticoagulant poisoning or overdose as obvious causes of haemorrhage. However, do not consider anticoagulant therapy, without mention of poisoning or overdose, as an obvious cause of haemorrhage. Further, consider treatment with steroid, aspirin, and nonsteroidal anti-inflammatory drugs (NSAIDs) as obvious causes of gastric haemorrhage ([ME24.Y](#)). Consider gastrointestinal haemorrhage ([ME24.9](#)) as an obvious cause of secondary ([3A00.0]) or unspecified anaemia ([3A9Z]).

2.19.2.19 Aspiration and inhalation

Consider conditions listed under Section [2.19.2.11 Pneumonia](#), as obvious causes of aspiration and inhalation [PB04.-PB07](#).

2.19.2.20 Surgery and other invasive medical procedures

Consider surgery or other invasive medical procedures, carried out within four weeks before death, as obvious causes of conditions that are considered common postprocedural complications. This applies also if the surgery or procedure is reported in a separate space on the certificate and not in Part 1 or Part 2. If there is no information about the time that elapsed between the surgery or other medical procedures and death consider as if it was carried out within four weeks before death.

A list of such conditions, with specific instructions, is given in Mortality Annex ([3.14.9 List of conditions to be considered obvious consequences of surgery and other invasive medical procedures](#)). If a condition that can be treated by surgery or other invasive medical procedures is reported on the certificate and surgery or a procedure of the same site is also

reported on the certificate, then assume that this condition was the cause of the surgery or procedure.

2.19.2.21 Common secondary conditions

Consider the following as the obvious cause of the common secondary conditions listed in the table below:

- [Wasting Diseases](#) (such as malignant neoplasms and malnutrition)
- [Diseases causing paralysis](#) (such as cerebral haemorrhage or thrombosis)
- other disease that limits the ability to care for oneself, including dementia and degenerative diseases of the nervous system
- [Serious injuries](#) (grade 1-4 according to the injury priority list in the Mortality Annex (3.14.5))

However, do not consider respiratory conditions as the obvious cause of such secondary conditions.

Conditions in categories flagged with an 'M' (Maybe) should be considered obvious consequences of wasting and paralysing conditions only if they meet the prerequisite for code assignment noted in the final column of the table.

Common secondary conditions

	Consequence	Maybe	Qualifier
3A00.0	Acquired iron deficiency anaemia due to blood loss		
3A9Z	Anaemias or other erythrocyte disorders, unspecified		
5B51	Wasting in infants, children or adolescents		
5B52	Acute malnutrition in infants, children or adolescents		
5B71	Protein deficiency		
5B7Z	Unspecified undernutrition		
5C70	Volume depletion		
8B40	Cauda equina syndrome		
8E45	Locked-in syndrome		
BB00	Pulmonary thromboembolism		
BD30.0	Acute upper limb arterial occlusion		
BD30.2	Acute lower limb arterial occlusion		
BD71.4	Lower limb deep vein thrombosis		
BD71.Y	Other specified deep vein thrombosis		
BD72	Venous thromboembolism		
DA91.31	Enterolith of small intestine		
DB30.3	Impaction of large intestine		
DB30.2	Acute mesenteric venous occlusion		
EH90	Pressure ulceration		
GB50	Acute tubulo-interstitial nephritis	M	Diseases causing paralysis or inability to control bladder
GB51	Acute pyelonephritis	M	Diseases causing paralysis or inability to control bladder
GB55.Y	Other specified chronic tubulo-interstitial nephritis	M	Diseases causing paralysis or inability to control bladder
GB55.Z	Chronic tubulo-interstitial nephritis, unspecified	M	Diseases causing paralysis or inability to control bladder
GB54	Tubulo-interstitial nephritis, not specified as acute or chronic	M	Diseases causing paralysis or inability to control bladder

	Consequence	Maybe	Qualifier
GB60-GB6Z	Kidney failure		
GB90.3	Ischaemia or infarction of kidney	M	The condition in GB90.3 must be specified as an embolism of the renal artery
GC00.1	Infectious cystitis	M	Diseases causing paralysis or inability to control bladder
GC00.3	Interstitial cystitis	M	Diseases causing paralysis or inability to control bladder
GC00.Z	Cystitis, unspecified	M	Diseases causing paralysis or inability to control bladder
GC01.4	Neuromuscular dysfunction of bladder, not elsewhere classified		
GC02.0	Urethral abscess	M	Diseases causing paralysis or inability to control bladder
GC02.1	Nonspecific urethritis	M	Diseases causing paralysis or inability to control bladder
GC02.Y	Other specified urethritis and urethral syndrome	M	Diseases causing paralysis or inability to control bladder
GC03	Urethral stricture	M	Diseases causing paralysis or inability to control bladder; Exclude post traumatic urethral strictures
GC08	Urinary tract infection, site not specified	M	Diseases causing paralysis or inability to control bladder
MB50-MB5Z	Paralytic symptoms		
ME05.0	Constipation		
MG20.Z	Cachexia, unspecified		

2.19.2.22 Secondary peritonitis

Consider the following conditions as obvious causes of Secondary peritonitis ([DC50.1](#)) and Peritonitis, unspecified ([DC50.Z](#)): [Secondary peritonitis and unspecified peritonitis](#)

2.19.2.23 Neutropenia

Consider the following conditions [Value set for selected neoplasms categories](#) :

- Neoplasms of central nervous system or related structures
- Neoplasms of haematopoietic or lymphoid tissues
- Malignant neoplasms, except primary neoplasms of lymphoid, haematopoietic, central nervous system or related tissues

as an obvious cause of Neutropenia ([4B00.0](#)) (causing Neutropenia directly or due to the treatment of these malignant neoplasms)

2.19.3 Special instructions on linkages and other provisions (Step M1)

Use the list in this section in Step M1.

The tentative underlying cause is listed in the left-hand column. If the conditions specified in the right-hand column apply, then use the code in bold as the new tentative underlying cause. There are two types of combination:

‘with mention of’ means that the other condition may appear anywhere on the certificate;

‘when reported as the cause of’ means that the other condition must appear in a correct causal relationship or be otherwise indicated as being due to the tentative underlying cause.

For some conditions, there are further requirements, for example that a specific term has been used either for the tentative underlying cause or for the condition that may change the underlying cause code.

2.19.3.1 Special instructions on Chapter 01 Certain infectious or parasitic diseases

TUC is:	when reported as the cause of:	code to:
[{01}(https://icd.who.int/browse/latest-release/mms/en#1435254666)]	2A00-2A0Z Neoplasms of central nervous system or related structures	2A00-2A0Z Neoplasms of central nervous system or related structures
	2A20-2B3Z Neoplasms of haematopoietic or lymphoid tissues	2A20-2B3Z Neoplasms of haematopoietic or lymphoid tissues
	2B50-2E2Z Malignant neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues	2B50-2E2Z Malignant neoplasms, except primary neoplasms of lymphoid, haematopoietic, central nervous system or related tissues
TUC is:	with mention of:	code to:
1A61 Early syphilis	1A62 Late syphilis	1A62 Late syphilis
TUC is:	with mention of:	code to:
1B10 Tuberculosis of the respiratory system	CA60 Pneumoconiosis	CA60.3 Pneumoconiosis associated with tuberculosis
TUC is:	with mention of:	code to:
1B11 Tuberculosis of the nervous system	1B10 Tuberculosis of the respiratory system	1B10 , unless 1B12 is reported as the cause of 1B10 and with a specified duration exceeding that of the condition in 1B10 Tuberculosis of the respiratory system
1B12 Tuberculosis of other systems and organs		
TUC is:	with mention of:	code to:

TUC is:	when reported as the cause of:	code to:
1C1C.2 Meningococcaemia	1C1C.0 Meningococcal meningitis	1C1C.0 Meningococcal meningitis
	1C1C.1 Waterhouse-Friderichsen syndrome	1C1C.1 Waterhouse-Friderichsen syndrome

TUC is:	with mention of:	code to:
1C41 Bacterial infection of unspecified site	1G40 Sepsis without septic shock	1G40 Sepsis without septic shock
1E1Z Unspecified viral disease	1G41 Sepsis with septic shock	1G41 Sepsis with septic shock

TUC is:	when reported as the cause of:	code to:
1E50.1 Acute hepatitis B	DB93 Hepatic fibrosis or cirrhosis	1E51.0Z Chronic hepatitis B, unspecified
1E50.2 Acute hepatitis C	DB99.8 Chronic hepatic failure	1E51.1 Chronic hepatitis C
1E50.3 Acute hepatitis D		1E51.2 Chronic hepatitis D
1E50.4 Acute hepatitis E		1E51.3 Chronic hepatitis E
1E50.Y Other specified acute viral hepatitis		1E51.Y Other specified chronic viral hepatitis
1E50.Z Acute viral hepatitis, unspecified		1E51.Z Chronic viral hepatitis, unspecified

TUC is:	with mention of:	code to:
1F2Z Mycoses, unspecified	1G40 Sepsis without septic shock	1G40 Sepsis without septic shock
1F5Z Unspecified protozoal disease	1G41 Sepsis with septic shock	1G41 Sepsis with septic shock
1H0Z Infection, unspecified		

Human immunodeficiency virus disease

TUC is:

[1A33](#) Cystoisosporiasis

[1D82](#) Cytomegaloviral disease

[1B21](#) Infections due to non-tuberculous mycobacteria

[1B2Y](#) Other specified mycobacterial diseases

[1B2Z](#) Mycobacterial diseases, unspecified

[1F00](#) Herpes simplex infections, **except** [1F00.Z](#) Herpes simplex infections, unspecified

[1F23.2](#) Candidosis of gastrointestinal tract

[1F23.31](#) Pulmonary candidosis

[1F25](#) Coccidioidomycosis

[1F27](#) Cryptococcosis

[1F2A](#) Histoplasmosis

[1F2G](#) Pneumocystosis

[1F57](#) Toxoplasmosis

[2A60.5](#) Blastic plasmacytoid dendritic cell neoplasm, **specified as** primary cerebral

[2A80](#) Follicular lymphoma, **specified as** primary cerebral

[2A81](#) Diffuse large B-cell lymphomas, **specified as** immunoblastic

[2A85](#) Other specified mature B-cell neoplasms or lymphoma, **specified as** primary cerebral

[2A85.6](#) Burkitt lymphoma including Burkitt leukaemia

[2A86](#) B-cell lymphoma, mixed features, **specified as** primary cerebral

with mention of:

[1C60-1C62.Z](#) *Human immunodeficiency virus disease*

[MA14.0](#) Laboratory evidence of HIV

code to:

[1C60-1C62.Z](#) *Human immunodeficiency virus disease*

Use additional code if desired, to specify the individual associated condition reported.

TUC is:

[2A90-2B2Z](#) Mature T-cell or NK-cell neoplasms, **specified as** primary cerebral (**except** [2B03](#))

[2B30](#) Hodgkin lymphoma, **specified as** primary cerebral

[2B33](#) Malignant haematopoietic neoplasms without further specification, **specified as** primary cerebral

[2C77](#) Malignant neoplasms of cervix uteri

[8A45.02](#) Progressive multifocal leukoencephalopathy

[8E47](#) Encephalopathy, not elsewhere classified

[CA40.20](#) Pneumonia due to pneumocystis

[MG20.Z](#) Cachexia, unspecified

with mention of:**code to:****TUC is:**

[1C60-1C62.Z](#) Human immunodeficiency virus disease

Note: Modes of dying, ill-defined conditions and conditions unlikely to cause death should not be linked to categories in [1C60-1C62.Z](#) Human immunodeficiency virus disease unless the coding tool guides you

Note: Use additional code if desired, to specify the individual associated condition reported.

TUC is:

[1C60-1C62.Z](#) Human immunodeficiency virus disease

with mention of:

[1C60-1C62.Z](#) Human immunodeficiency virus disease, of a more severe stage with a fifth digit greater (except Z)

code to:

[1C60-1C62.Z](#) Human immunodeficiency virus disease, of a more severe stage use the highest fifth digit

TUC is: 1B10-1B1Z Tuberculosis	with mention of: 1C62 Human immunodeficiency virus disease without mention of tuberculosis or malaria	code to: 1C62.0 to 1C60.0 ; 1C62.1 to 1C60.1 ; 1C62.2 to 1C60.2 ; 1C62.3 to 1C60.3Z ; 1C62.Z to 1C60.Z ; MA14.0 to 1C60.Z Regardless of the subcode (5th to 6th character) that the TUC has, code to 1C60 Human immunodeficiency virus disease associated with tuberculosis (respecting the 5th or 6th character if present). If the code in the second column is MA14.0 code as 1C60.Z Human immunodeficiency virus disease associated with tuberculosis, clinical stage unspecified
1G80 Sequelae of tuberculosis	MA14.0 Laboratory evidence of human immunodeficiency virus	
1C62 Human immunodeficiency virus disease without mention of tuberculosis or malaria MA14.0 Laboratory evidence of human immunodeficiency virus	1B10-1B1Z Tuberculosis	1C60 Human immunodeficiency virus disease associated with tuberculosis
TUC is: 1F40-1F4Z Malaria	with mention of: 1C62 Human immunodeficiency virus disease without mention of tuberculosis or malaria MA14.0 Laboratory evidence of human immunodeficiency virus	code to: 1C61 Human immunodeficiency virus disease associated with malaria
1C62 Human immunodeficiency virus disease without mention of tuberculosis or malaria MA14.0 Laboratory evidence of human immunodeficiency virus	1F40-1F4Z Malaria	1C61 Human immunodeficiency virus disease associated with malaria

TUC is:

[2B57](#) Kaposi
sarcoma,
primary site

with mention of:

[1C60-1C62.Z](#) Human
immunodeficiency virus
disease

[MA14.0](#) Laboratory
evidence of HIV

code to:

[1C60.30](#) Kaposi sarcoma associated with
human immunodeficiency virus disease
associated with tuberculosis

[1C61.30](#) Kaposi sarcoma associated with
human immunodeficiency virus disease
associated with malaria

[1C62.30](#) Kaposi sarcoma associated with
human immunodeficiency virus disease
without mention of tuberculosis or malaria

TUC is:	when reported as the cause of:	code to:
[{03}(https://icd.who.int/browse/latest-release/mms/en#1766440644)] Chapter 3 'Diseases of the blood or blood-forming organs'	1C60-1C62.Z Human immunodeficiency virus disease, and where the certificate indicates that the HIV disease is a result of a blood transfusion given treatment for the originating condition	1C60-1C62.Z Human immunodeficiency virus disease

[4A00-4A0Z](#) Primary immunodeficiencies

[4A20](#) Acquired immunodeficiencies

[4B00-4B0Z](#) Immune system disorders involving white cell lineages

[4B20-4B2Y](#) Certain disorders involving the immune system

[4B4Y](#) Other specified diseases of the immune system

[4B4Z](#) Diseases of the immune system, unspecified

2.19.3.2 Special instructions on Chapter 02 Neoplasms

TUC is:	when reported as the cause of:	code to:
2E60-2E6Z In situ neoplasms, except of lymphoid, haematopoietic, central nervous system or related tissues	metastatic spread, or if it is clear from other information on the certificate that the in situ neoplasm caused metastatic spread If there is no indication that the in situ neoplasm caused metastatic spread,	the corresponding primary malignant neoplasm Consider the in situ neoplasm unlikely to cause death, and follow the instructions in Step SP8 Conditions unlikely to cause death.

2.19.3.3 Special instructions on Chapter 03 Diseases of the blood or blood-forming organs

2.19.3.4 Special instructions on Chapter 04 Diseases of the immune system